IRT / RTSM User Acceptance Testing

Operations-only · synthetic · no PHI · say **participant** not **subject**

What UAT confirms

In a test environment you confirm that the vendor's randomization and drug-supply build does **exactly** what your URS and specs say, before any real participant is randomized. You operate the staging IRT in a browser, record dated screenshot evidence, and transcribe results into your workbook so the record is attributable and stands alone for an auditor.

The go-live gate (conjunctive)

Sign-off is READY only when **all three** hold:

- Every **Critical** test passes
- **0** open Critical / Major defects
- **100%** traceability (every requirement has a test)

Current snapshot: **NOT ready - blocked**.

The gate packages evidence; a named human signs. It does not grant approval.

Live snapshot

Test scripts	**29**
Passed	**22**
Failed	**2**
Open defects	**3**
Requirements traced	**97%**

Defect severity drives the gate

Critical	Could break the blind, mis-randomize, mis-dose, or corrupt integrity	Blocks
Major	Significant functional error in a key workflow (resupply, wrong kit, missing alert)	Blocks until fixed
Minor	Cosmetic / non-functional (label typo, missing site number)	Fix or waive

The 8-step workflow

1. Derive test cases from URS + specs (positive · negative · boundary)

2. Load the approved test list into the staging IRT

3. Execute each case in the browser; be systematic, not happy-path

4. Record result + dated screenshot evidence

5. Log every failure as a defect with a severity

6. Vendor fixes; retest against the new build

7. Traceability: confirm every requirement maps to a passing test

8. Evaluate the gate; a named human signs the package into the eTMF

Run it

`shiny::runApp('uat_irt/shiny')` Mark Pass/Fail, close a defect, and the KPIs + gate recompute live.